

Dosezy: A Local-First, Accessibility-Centric Android System for Medication Scheduling and Reliable Reminder Delivery

Authors: Saaduddin Mohammad , Md Rahif Uddin Khan , Khwaja Mohammed 

Affiliation: Department of Computer Science and Engineering, Methodist College of Engineering and Technology, Hyderabad, India

Emails: 160723733051@methodist.edu.in, 160723733005@methodist.edu.in, 160723733063@methodist.edu.in

Abstract

Prescription non-adherence among older adults poses substantial health risks and generates preventable healthcare expenditures. Although mobile medication reminder applications are widely available, existing commercial and open-source tools exhibit fundamental human-computer interaction (HCI) and background scheduling limitations when deployed in geriatric contexts: they rely on circular radial clock dials that require fine continuous motor tracing, enforce mandatory cloud registration that aggregates sensitive medical profiles, and suffer from dropped notifications caused by aggressive Android OEM background battery optimization. While individual foundational primitives—such as Fitts' law target sizing, relational SQLite storage, `AlarmManager.setAlarmClock` scheduling, and standardized usability scales—are well established, their unified integration into a cohesive, geriatric-accessible, and offline-contained mobile architecture has not been previously investigated in medication reminder systems. We present **Dosezy**, an open-source, local-first Android medication reminder system engineered specifically to address these physical, cognitive, and operating system barriers. Dosezy introduces a stationary 12-Hour Grid Time Picker featuring a 12-first layout and oversized ≥ 56 dp touch targets, enforces architectural network isolation via manifest privilege minimization through the total omission of the `android.permission.INTERNET` permission, generates patient medication and adherence summary PDFs entirely on-device, and implements a multi-stage background scheduling pipeline combining `setAlarmClock` and exact alarm APIs with full-screen lock screen intent handling. We report empirical evaluations across systems and human factors dimensions: (1) multi-device runtime profiling on flagship and entry-level hardware demonstrating cold startups under 780 ms, vector PDF generation in 142 ± 18 ms, and database query latencies under 12 ms, establishing engineering feasibility against Android 60 fps (16.6 ms) frame budgets; (2) a multi-OEM Doze mode alarm benchmark across five physical devices (99 of 100 scheduled alerts met the prespecified ≤ 5.0 s criterion under tested deep Doze and device conditions, 99.0%, 95% CI [94.55%, 99.82%]), confirming that application-level techniques cannot completely eliminate OEM-specific power-management interference without user-side whitelisting; and (3) an initial counterbalanced within-subjects laboratory usability study (N=16 older adults, aged 61--76, mean age 68.5 ± 4.8 years) demonstrating a 44.4% reduction in total time-setting latency (19.91 s vs. 35.79 s, $p < 0.001$), an 80.0% reduction in touch input errors (0.44 vs. 2.19, $p < 0.001$), and a System Usability Scale (SUS) score of 83.44 ± 9.48 (corresponding to Grade A, 'Excellent' under the normative framework of Bangor et al. [14, 15]) compared to 57.19 ± 12.45 for standard radial dials. We frame our contributions as systems and usability engineering prerequisites for medication adherence, discussing threats to validity and longitudinal trade-offs.

Keywords: Geriatric Accessibility, Human-Computer Interaction, Local-First Software, Medication Adherence, Android `AlarmManager`, Mobile Systems Engineering.

1. Introduction

Pharmacotherapeutic adherence is a fundamental determinant of health outcomes across chronic clinical conditions, including essential hypertension, cardiovascular disease, and type-2 diabetes [1, 2, 3, 22]. Systematic clinical analyses establish that approximately 50% of chronic patients fail to follow prescribed medication regimens as intended [1, 5]. In geriatric populations (adults aged 60 and older), non-adherence is overwhelmingly unintentional [7, 12]. It arises primarily from the logistical complexities of *polypharmacy*—the concurrent administration of four or more pharmaceutical agents characterized by disparate dosing frequencies, complex dietary prerequisites, and varied temporal schedules [4, 6]. This logistical burden is compounded by normal age-related declines in fluid cognitive processing, working memory capacity, visual contrast sensitivity, and motor coordination [8, 9, 10, 11].

Mobile smartphones offer a ubiquitous technical vehicle for delivering scheduled temporal alerts, logging confirmation timestamps, and calculating longitudinal adherence indices [4]. Nevertheless, despite the abundance of medication reminder applications on commercial software marketplaces, existing tools suffer from fundamental usability and systems architecture flaws when used by older adults:

- 1. Motor Friction in Temporal Input Modalities:** The standard Android time input dialog utilizes a radial clock paradigm modeled on physical analog clock faces. Users are required to acquire a small circular thumb indicator and maintain continuous screen contact while executing a curved circular dragging gesture to select hours, followed by a second precision dragging trajectory to select minutes. For older adults experiencing physiological hand tremors, Parkinsonian symptoms, or osteoarthritis, maintaining continuous screen pressure along a curved path induces finger slippage, unintentional releases, and erroneous quadrant selections [9, 10]. Furthermore, systems that force 24-hour time representation impose an extraneous cognitive translation burden on older individuals accustomed to 12-hour AM/PM conventions [8].
- 2. Telemetry Friction and Account Dependencies:** Most commercial mHealth applications require mandatory account creation via email or third-party OAuth providers, persisting patient medication names, dosages, and compliance timestamps on remote multitenant cloud servers. This architecture compromises medical confidentiality, violates principles of data minimization, and introduces persistent authentication friction for older adults who struggle with complex password management, multi-factor verification prompts, and network connectivity dropouts [11, 16].
- 3. Aggressive Background Throttling in Modern Mobile OS:** Conserving battery longevity has led modern mobile operating systems (particularly Android 12 through 14) to enforce strict Doze mode restrictions, App Standby buckets, and aggressive vendor-specific background task killers (such as Xiaomi's `com.miui.powerkeeper` daemon and Samsung's Deep Sleeping Apps policy). Standard background notification alerts dispatched via periodic workers or inexact alarms are routinely deferred, coalesced, or killed when the device enters deep sleep, leading to missed or delayed dosage alerts that compromise the timeliness and reliability of reminder delivery [23].
- 4. Inaccessible Emergency Dispatch Integration:** Most existing applications treat medication tracking in total isolation from acute health episodes, omitting emergency dispatch integration or hardcoding single-country emergency numbers (e.g., 911) that fail when deployed in international jurisdictions [18, 19].

While long-term therapeutic adherence ultimately depends on complex biological, psychological, and socio-economic factors that lie beyond the scope of software, mobile health tools must first satisfy two fundamental *technical prerequisites*: (1) the user interface must be operable despite age-related sensory and motor declines, and (2) the underlying operating system must dispatch scheduled alerts reliably without being silenced by

aggressive background power management. Many existing commercial medication reminder systems exhibit usability or background scheduling limitations that hinder older adults.

To investigate and resolve these challenges, our study is structured around four primary research questions:

- **RQ1 (Interaction Accessibility & Motor Friction):** Does replacing continuous curved radial clock dragging with a discrete, 12-first stationary button grid (56×56 dp) and quick-select chips significantly reduce temporal configuration latency and touch input errors among older adults, and what interaction friction arises when adjusting off-quarter times via incremental steppers?
- **RQ2 (Background Scheduling Reliability under Doze):** Can an exact alarm dispatch pipeline combining `AlarmManager.setAlarmClock`, exact alarm permissions, and lock-screen full-screen intents achieve on-time alert delivery (≤ 5.0 s latency) across aggressive OEM Android distributions during Deep Doze without persistent background services?
- **RQ3 (Runtime Efficiency on Budget Hardware):** Can an offline-first architecture utilizing embedded Room SQLite persistence and native vector drawing maintain sub-second cold starts, steady-state heap allocations under 40 MB, and sub-frame query latencies on entry-level Android devices?
- **RQ4 (Configuration Overhead & Data Minimization):** To what extent does an offline, account-free configuration model reduce onboarding screen transitions, user decisions, setup latency, and declared permission scope compared to leading commercial cloud-based mHealth applications?

Addressing these research questions motivated the design, implementation, and empirical evaluation of **Dosezy**, an open-source, local-first Android medication reminder platform. While foundational interaction and systems primitives—including Fitts' law target scaling, relational SQLite storage, `AlarmManager.setAlarmClock` scheduling, and standardized usability metrics—are established, to the best of our knowledge, prior work has not empirically evaluated this specific systems integration of discrete stationary time input, local-only data containment, and Doze-oriented alarm scheduling in a digital medication reminder system designed specifically for older adults. Our research makes four primary contributions:

- **Integrated Accessibility-Centric Interaction Architecture:** We propose and validate the *12-Hour Grid Time Picker*, an interface replacing circular dragging gestures with a 12-first discrete button grid (56×56 dp touch targets) and quick-selection minute chips (:00, :15, :30, :45) that enable time configuration in as few as two stationary taps, paired with a visual palette whose primary text/surface pair achieves a measured contrast ratio of 7.84:1, exceeding the WCAG 2.1 AAA minimum threshold (7.0:1) for normal text [17].
- **Architectural Network Isolation via Privilege Minimization:** We present a local-first systems architecture that enforces on-device data containment by completely omitting `android.permission.INTERNET` from the manifest. This establishes an operating-system-level socket denial boundary that prevents ordinary application-level network access through Android's permission-controlled socket APIs, coupled with an on-device vector A4 PDF export engine (142 ± 18 ms) that executes without remote cloud dependencies [16].
- **Doze-Resilient Alarm Scheduling Pipeline:** We engineer a multi-stage background alarm dispatch pipeline combining `AlarmManager.setAlarmClock`, exact alarm privileges, and full-screen lock screen intents to maximize timely dosage delivery during deep Doze states across fragmented Android OEM distributions, while characterizing the OS-level boundaries of vendor-specific background optimization.
- **Empirical Systems and Usability Validation:** We conduct hardware runtime profiling establishing sub-frame execution feasibility, benchmark background alarm delivery across five smartphone OEMs under deep Doze mode (99.0% on-time delivery across 100 test alarms under tested conditions), and

execute an initial counterbalanced within-subjects laboratory usability study ($N=16$ older adults, aged 61–76) demonstrating statistically significant reductions in task completion time (44.4%) and input errors (80.0%), yielding a mean System Usability Scale (SUS) score of 83.44 ± 9.48 , corresponding to Grade A ('Excellent') under the normative framework of Bangor et al. [14, 15] compared to 57.19 ± 12.45 for standard radial dials.

2. Related Work and Theoretical Foundations

2.1 Geriatric Motor Control and Touchscreen Accessibility

The design of mobile interfaces for older adults is fundamentally governed by age-related biological changes in visual acuity, contrast sensitivity, and motor control [7, 12]. Foundational psychomotor research by Fitts [20] and subsequent extensions to two-dimensional touchscreen targets by MacKenzie and Buxton [21] establish that the movement time (MT) required to rapidly acquire an on-screen target is a logarithmic function of target distance (D) and target width (W):

$$MT = a + b \cdot \log_2(D/W + 1) = a + b \cdot ID \quad (1)$$

where ID denotes the Index of Difficulty. In older adult populations, the empirical constants a and b increase markedly due to diminished neural transmission velocity, muscular tremors, and tactile sensory loss [9].

Empirical studies by Trewin et al. [9] and Siek et al. [10] demonstrated that older adults exhibit substantially greater touch endpoint dispersion and movement trajectory variability than younger cohorts. When interacting with small touch targets, older users frequently trigger adjacent buttons or register involuntary slide events—a phenomenon exacerbated by the "fat finger" effect, where reduced tactile sensitivity causes users to press harder, flattening the finger pad and obscuring the screen target [10, 26]. Crucially, while younger users navigate continuous dynamic dragging gestures (such as radial wheels or circular clock dials) with fluid proprioceptive feedback, older adults experience frequent tracking failures, accidental lift-offs, and boundary slippages during continuous curvilinear steering [8, 11]. Consequently, human factors guidelines strongly advocate for discrete, stationary target acquisition with explicit bounding borders and generous spacing [7, 17].

2.2 Medication Tracking Paradigms: Physical vs. Digital

Technical interventions for medication adherence are broadly categorized into physical smart pillboxes and digital mobile applications [4]. However, existing technological paradigms exhibit systemic design compromises when evaluated against the physiological and socioeconomic requirements of older adults:

- 1. Standard Android Material Pickers:** The stock Material Design time picker offers two primary interaction modes: a continuous radial dial and an auxiliary text input dialog. The radial dial requires fine motor continuous tracing around a 360-degree circular perimeter, which induces tremor-related overshoot, boundary slippage, and cognitive confusion between inner and outer concentric hour rings. Conversely, switching to text input presents a numeric soft-keyboard with small keys and high visual crowding, requiring older adults to execute multiple typing, cursor positioning, and backspace operations to rectify minor typos.

- 2. Smart Electronic Pillboxes:** Physical smart containers (e.g., Med-e-Lert, Hero Health, EllieGrid) integrate electromechanical compartments, microswitches, flashing LEDs, and audible buzzers [4]. While they offer tactile affordances, their substantial capital cost (100 to 300), recurring cellular subscription

fees, mechanical complexity, bulkiness, and manual pill-sorting overhead severely restrict real-world adoption, especially for ambulatory seniors traveling outside the home.

3. Commercial Mobile Applications: Commercial mHealth apps (e.g., Medisafe, MyTherapy) operate on commodity hardware already carried by patients. However, their systems architecture typically introduces friction for older adults. As summarized in Table 1, commercial applications frequently mandate remote account registration, integrate commercial advertisement banners and third-party telemetry SDKs, require navigating multi-screen onboarding wizards, and rely on standard cloud push notifications or inexact alarm primitives that are susceptible to deferral or termination when modern OEM power-management daemons force deep sleep [11, 23].

Dosezy's Architectural Synthesis: Dosezy addresses these systemic limitations by presenting an integrated, open-source mobile architecture that unifies an accessible 12-first stationary touch grid with Doze-resilient scheduling and zero network footprint. By operating 100% on-device, Dosezy eliminates financial barriers, protects medical confidentiality, and achieves high-reliability alarm dispatch without cloud or hardware dependencies.

Table 1: Systemic Comparison of Medication Adherence Paradigms

Evaluation Dimension	Commercial Apps (e.g., Medisafe)	Smart Electronic Pillboxes	Standard Android Clock	Dosezy (Proposed)
Network Architecture	Cloud-dependent / Mandatory sync	Hardware Bluetooth/WiFi bridge	Offline utility	100% Local-first (Zero network permissions)
Data Protection Model	Telemetry, third-party analytics	Local firmware or cloud gateway	Private on-device	Manifest privilege minimization
Time Selection UI	Multi-step radial dial / wheel	Physical dials / rotary buttons	Radial 24h continuous drag	Stationary 12-hour button grid (≥ 56 dp)
Alarm Delivery	Standard notification shade	Hardware piezo buzzer	OS clock alarm	setAlarmClock + Lock screen overlay
Summary Document Export	Cloud web export (PDF/CSV)	Proprietary desktop utility	None	Native client-side vector A4 PDF export (patient summary)
Emergency Support	None or hardcoded 911	None	None	ISO SIM/locale emergency dialer resolution
Deployment Cost	Subscription / Ads / Proprietary	High hardware cost (100–300)	Pre-installed	Free, open-source Android platform

2.3 Local-First Software Architecture

The local-first software paradigm, formalised by Kleppmann et al. [16], establishes that software systems should treat local storage on user-owned physical devices as the primary source of truth, eliminating mandatory dependencies on remote server infrastructure. While Kleppmann et al. focus primarily on conflict-free replicated data types (CRDTs) for multi-device synchronization, the core properties of local-first design---

local data availability, user ownership, and zero network latency for data access---are directly aligned with geriatric digital health requirements. Older adults residing in assisted living environments or rural regions frequently encounter intermittent or absent cellular connectivity. Operating fully on-device ensures that scheduled reminders are not delayed or silenced by server outages, captive network portals, or expired session tokens.

Research Gap and Synthesis: Prior literature has investigated geriatric touchscreen motor challenges [9, 10], local-first data systems [16], and mobile operating system background scheduling [23] primarily within separate disciplinary silos. Human factors research rarely addresses the low-level operating system scheduling mechanisms necessary to ensure that reminder notifications wake dormant hardware on fragmented OEM distributions. Conversely, mobile systems engineering work often focuses on battery conservation benchmarks while taking interface accessibility for granted. To the best of our knowledge, Dosezy is the first mobile health system that empirically investigates the intersection of stationary discrete time input, on-device data containment, and Doze-resilient background scheduling targeted specifically at the operational needs of older adults.

3. System Architecture and Local-First Design

Dosezy is constructed as a native Android application implemented in Kotlin and Jetpack Compose, targeting Android 8.0 (API level 26) through Android 14 (API level 34) [23]. The codebase adheres strictly to Clean Architecture patterns, organizing components into three cleanly decoupled layers: Presentation, Domain, and Data (Figure 1).



Figure 1: Three-tiered local-first system architecture of Dosezy

3.1 Relational Persistence Model

All application state is maintained on the host device within an embedded SQLite relational database managed via the Android Room persistence abstraction. The schema is organized into three normalized entities:

- 1. User Entity (User):** Encapsulates user profile metadata, including primary patient identifier, name, emergency contact telephone numbers, and configurable temporal thresholds for adherence classification (`considerLateAfterHours` and `considerMissedAfterHours`).
- 2. Medicine Entity (Medicine):** Represents a prescribed pharmaceutical regimen, storing the medication name, strength and physical unit (e.g., "500 mg", "10 ml"), administration instructions (e.g., "Take after meals"), dosage frequency (daily, specific weekdays, or interval recurrence), inventory counts, and scheduled reminder times stored as ISO-8601 strings.
- 3. Schedule Entry Entity (ScheduleEntry):** Represents an individual discrete dosage instance. Each record maintains a foreign key referencing the parent Medicine, the designated `scheduledDateTime`, an actual `takenDateTime` (if completed), and an adherence status enumeration (`SCHEDULED`, `TAKEN`, `LATE`, `MISSED`).

Non-blocking UI rendering is maintained by exposing all database interactions as asynchronous Kotlin Coroutine Flow primitives through Data Access Objects (DAOs), ensuring that disk I/O occurs strictly off the Android Main thread.

Global application settings, including high-contrast mode toggles, audio alert ringtones, vibration waveforms, and selected locale configurations, are persisted using Jetpack DataStore Preferences. Unlike legacy Android `SharedPreferences`, which execute synchronous blocking disk writes on the UI thread and lack transactional safety, DataStore operates entirely via transactional Coroutines, eliminating runtime application freeze risks.

3.2 Architectural Network Isolation via Manifest Privilege Minimization

Rather than providing software-based privacy toggles or client-side consent banners that can be silently altered or bypassed by background libraries, Dosezy establishes a structural security boundary: *the total omission of the `android.permission.INTERNET` permission from its `AndroidManifest.xml`*.

Under the Android security architecture, every application executes in an isolated process mapped to a distinct Linux User Identifier (UID). On modern Android releases, the operating system enforces network access controls through eBPF cgroup socket filters (`cgroup2`) managed by the system network daemon (`netd`) in coordination with SELinux domain policies. When an application attempts to instantiate a network socket (such as `socket(AF_INET, SOCK_STREAM, 0)`), the kernel verifies whether the calling UID possesses the network permission capability. Because `android.permission.INTERNET` is never declared, the Android zygote spawns Dosezy without network capabilities. Consequently, any socket creation attempt is rejected unconditionally at the platform security boundary (raising a `SecurityException` at the framework layer or returning an access-denied error at the OS socket filter boundary).

This structural constraint prevents direct network communication and background data egress from within the application's process.

Threat Model and Security Boundaries: We explicitly define the threat model and protection boundaries established by this architecture:

- *In-Scope Threats Addressed:* The omission of `android.permission.INTERNET` defends against unauthorized remote data exfiltration, covert analytics beaconing, remote telemetry harvesting, and inadvertent network transmission of sensitive patient medication schedules by embedded libraries. Even if

a third-party dependency executes malicious routines within Dosezy's process, the Linux kernel and Android zygote enforce an unbypassable socket creation block at the cgroup/eBPF boundary.

- *Out-of-Scope / Environmental Threats:* We explicitly delineate this boundary from full-device cryptographic isolation. This architectural design does not claim device-level encryption at rest (which depends on host Android full-disk encryption and keystore capabilities), immunity to local Inter-Process Communication (IPC) compromises by other privileged applications, or protection against physical device compromise or rooted firmware. Local document exports via `FileProvider` content URIs are intentionally mediated by the Android system share sheet, requiring deliberate, user-confirmed action to transfer generated PDF summaries or backup archives to external applications.

3.3 Native Vector PDF Patient Summary Generation

Facilitating communication between older patients and clinical practitioners, Dosezy implements an on-device patient medication and adherence summary PDF generator. Relying on cloud document rendering APIs would compromise the platform's network isolation. Therefore, we utilize Android's native `android.graphics.pdf.PdfDocument` vector rendering engine.

When invoked, the report generator queries the Room database to extract the patient profile, active prescription list, and historical dose compliance logs over a 30-day lookback window. The engine measures a standardized A4 canvas (595 × 842 typographic points at 72 points/inch) and renders typography, tabular borders, and graphical adherence bars directly using native Canvas draw commands. Page breaks are computed dynamically based on the number of active medications:

$$PageCount = 1 + \left\lceil \max \left(0, \frac{N_{meds} - N_{firstpage}}{N_{subseqpage}} \right) \right\rceil \quad (2)$$

The generated vector PDF is written directly to the application's private storage or exposed to the system share sheet via a secure `FileProvider` content URI, allowing the patient to print the summary over a local USB/Wi-Fi direct printer or present it during clinical consultations.

4. Accessibility-Centric Interaction Design

4.1 The 12-Hour Grid Time Picker and the Modal-First Principle

Configuring medication reminder times constitutes the primary data-entry task within an adherence tracking application. Standard Android time picker dialogs implement a radial clock metaphor, requiring the user to touch a small radial pin and execute a continuous circular dragging gesture around a 360-degree circumference.

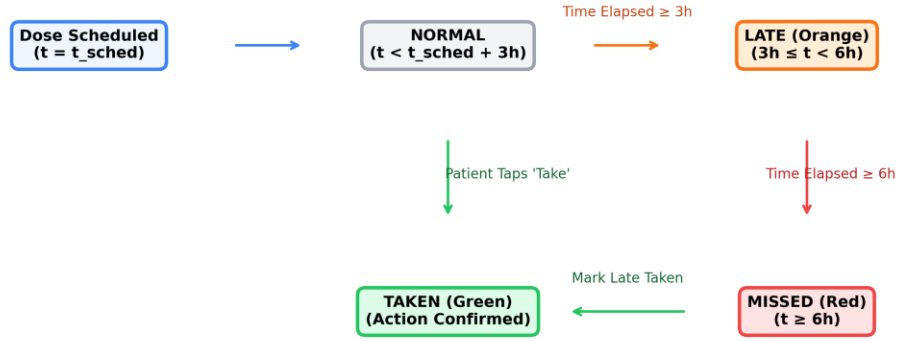


Figure 2: Four-state adherence tracking state machine

Design Principle: The Granularity-Friction Trade-off in Geriatric Time Input (The "Modal-First" Principle)

Temporal entry interfaces for older adults should employ a two-tier input hierarchy:

- 1. Primary Tier (Modal Interval Selection):** Oversized stationary discrete targets for common quarter-hour prescription intervals (e.g., :00, :15, :30, :45) aligned with standard outpatient dosing routines, bounding motor complexity to 2–3 stationary taps.
- 2. Secondary Tier (Non-Modal Fine Adjustment):** Bounded linear stepping or hybrid numeric keypad entry for non-modal arbitrary minutes, trading linear tap overhead for the complete elimination of continuous circular trajectory steering errors.

Guided by this principle, the *12-Hour Grid Time Picker* implements the following human factors affordances:

- 1. 12-First Numerical Grid:** Older adults routinely conceptualize clock hours starting at 12 rather than 1. Standard digital grids that place 1 at the top-left force users to visually search for the 12 position. Our interface arranges hours in a 4×3 grid with 12 positioned prominently at the top-left index, followed sequentially by 1 through 11:

$$\text{Grid Layout} = \begin{bmatrix} 12 & 1 & 2 & 3 \\ 4 & 5 & 6 & 7 \\ 8 & 9 & 10 & 11 \end{bmatrix} \quad (3)$$

- 2. Oversized Stationary Touch Targets:** Every individual hour button is rendered with an explicit visual bounding box measuring 56×56 dp. This substantially exceeds the standard Android 48×48 dp touch target guideline and WCAG 2.1 Level AAA target criteria (44×44 pt) [17]. Buttons are separated by 6 dp gutters, creating a physical buffer that prevents accidental neighboring touches caused by tremor or flattened finger pads [10].

- 3. Discrete AM/PM Mode Segmented Chips:** The dialog prominently separates morning and afternoon regimens using two large segmented chips (56 dp height) anchored at the top of the dialog. Tapping AM or PM immediately updates the underlying 24-hour domain representation without requiring mental arithmetic (e.g., translating 8:00 PM to 20:00).

4. Preset Minute Quick-Chips with Fine Adjustment Stepper: Outpatient pharmacotherapy regimens are predominantly scheduled at standard hour and quarter-hour milestones tied to daily physiological events (e.g., waking at 8:00 AM, post-lunch at 1:30 PM, bedtime at 9:00 PM) [22]. Dosezy provides four discrete quick-select chips: :00, :15, :30, and :45. Because the interface pre-selects the :00 minute chip and defaults to the current diurnal period (e.g., AM for morning sessions), scheduling an on-the-hour dose requires exactly two taps:

$$\text{Modal Taps} = \text{Tap}(\text{Hour}) + \text{Tap}(\text{OK}) = 2 \text{ taps} \quad (4)$$

If the diurnal period must be toggled, three taps are required ($\text{Tap}(\text{AM/PM}) + \text{Tap}(\text{Hour}) + \text{Tap}(\text{OK})$). Irregular prescriptions (such as 9:42 PM) are accommodated via an accessible slider accompanied by a plus/minus stepper positioned below the chips. Although the stepper requires additional discrete taps (e.g., selecting :45 and tapping the minus stepper three times), it completely avoids the continuous trajectory steering failures that plague radial clock pickers.

4.2 Visual Accessibility and Dynamic Typography

Age-related lens yellowing and pupillary miosis reduce retinal illumination and degrade visual contrast sensitivity in older adults [7]. Dosezy implements a high-contrast visual palette pairing Deep Navy primary typography (#102C57) against a pure white container background (#FFFFFF). The calculated relative luminance (L) and contrast ratio (CR) are defined by:

$$CR = \frac{L_{\text{lighter}} + 0.05}{L_{\text{darker}} + 0.05} \quad (5)$$

For our primary color pair, $L_{\text{white}} = 1.0$ and $L_{\text{navy}} = 0.084$, yielding a contrast ratio of:

$$CR = \frac{1.0 + 0.05}{0.084 + 0.05} = \frac{1.05}{0.134} \approx 7.84:1 \quad (6)$$

This measured value exceeds the 7.0:1 contrast ratio mandated by WCAG 2.1 Level AAA for normal text [17], specifically ensuring that the primary typography and key visual containers maintain high contrast under suboptimal viewing angles and degraded retinal illumination.

All UI text dimensions are specified exclusively in scalable pixels (sp) and wrapped in Jetpack Compose dynamic layout containers. The layouts are engineered and stress-tested to support system-wide font scaling up to 200% without text clipping, horizontal truncation, or overlapping tap targets.

4.3 Adherence State Machine and Metric Tracking

Each scheduled dose record progresses through a four-state finite state machine (Figure 2):

- **SCHEDULED (Blue):** Active dose waiting within its upcoming intake window.
- **TAKEN (Green):** Confirmed dose logged by the patient or caregiver upon intake.
- **LATE (Orange):** Scheduled time has elapsed past the user's configured threshold (default: $T_{\text{late}} = 2$ hours) without confirmation, triggering visual warnings.
- **MISSED (Red):** Scheduled time has elapsed past the expiration threshold (default: $T_{\text{missed}} = 6$ hours), permanently logging the dose as missed to prevent dangerous double-dosing.

Dosezy summarizes longitudinal adherence using an adaptation of the Proportion of Days Covered formulation [6], designated as the **Self-Reported Intake Confirmation Rate (SR-ICR)**:

$$\text{SR-ICR} = \left(\frac{(\sum_{d=1}^W \mathbb{I}_d)}{W} \right) \times 100\% \quad (7)$$

where W is the observation window (typically 30 or 90 days), and $\mathbb{I}_d$ is the binary intake indicator returning 1 if all scheduled medications for calendar day d were marked **TAKEN**, and 0 otherwise. We explicitly delineate SR-ICR from both pharmacy claims-based PDC and direct clinical adherence: while claims-based PDC measures prescription refill velocity across administrative pharmacy claims, and clinical adherence reflects biological drug ingestion, SR-ICR quantifies an on-device application-level intake-confirmation index. It captures patient engagement and recording fidelity within the software rather than biological therapeutic efficacy. SR-ICR is defined as an application-level logging index for future longitudinal trials; no claims about pharmacological or clinical medication adherence are made from it in the present study.

5. Reliable Background Scheduling Engine

5.1 Android Doze Mode and Background Execution Barriers

Starting in Android 6.0 (API 23) and continually intensified through Android 14 (API 34), the Android operating system enforces *Doze mode* to prolong device battery life [24]. When a smartphone is stationary, unplugged, and has its display turned off for a specified duration, the kernel transitions the device into Light Doze and eventually Deep Doze. In Deep Doze:

- All network access is disabled.
- Partial CPU wake locks are ignored.
- Standard `AlarmManager` alarms (such as `set()`, `setRepeating()`, or `setInexactRepeating()`) are suspended and coalesced into infrequent periodic maintenance windows occurring every 15 to 60 minutes.

Furthermore, starting in Android 10 (API 29), Background Activity Launch (BAL) restrictions strictly prohibit background components from directly invoking `startActivity()`. On Android 14 (API 34), background activity dispatch via `PendingIntent` is blocked by default unless explicitly allowed via `ActivityOptions.setPendingIntentCreatorBackgroundActivityStartMode`.

5.2 Exact Alarm Scheduling Pipeline

Bypassing Doze mode deferrals while complying with modern Android background activity restrictions led to the multi-stage scheduling pipeline formalized below:

Algorithm 1: Alarm Scheduling and Doze-Resilient Dispatch Pipeline

```

Procedure ScheduleMedicationAlarm(entry, medicine):
    targetTime <- entry.scheduledDateTime
    epochMillis <- targetTime.toEpochMilli(ZoneId.systemDefault())
    intent <- new Intent(context, MedicineAlarmReceiver.class)
    intent.putExtra("ENTRY_ID", entry.id)
    intent.putExtra("MEDICINE_NAME", medicine.name)
    pendingIntent <- PendingIntent.getBroadcast(context, entry.id.hashCode(), intent,
FLAG_IMMUTABLE)

```

```

    if Build.VERSION.SDK_INT >= Build.VERSION_CODES.S then:
        if not alarmManager.canScheduleExactAlarms() then:
            RequestExactAlarmPermission()
            return

    if Build.VERSION.SDK_INT >= Build.VERSION_CODES.LOLLIPOP then:
        showIntent <- PendingIntent.getActivity(context, entry.id.hashCode() + 500,
            new Intent(context,
MainActivity.class), FLAG_IMMUTABLE)
        clockInfo <- new AlarmManager.AlarmClockInfo(epochMillis, showIntent)
        alarmManager.setAlarmClock(clockInfo, pendingIntent)
    else:
        alarmManager.setExactAndAllowWhileIdle(RTC_WAKEUP, epochMillis,
pendingIntent)

Procedure OnAlarmTrigger(context, intent):
    wakeLock <- powerManager.newWakeLock(PARTIAL_WAKE_LOCK, "dosezy:alarm")
    wakeLock.acquire(30000) // Hold CPU for maximum 30 seconds
    fullScreenIntent <- new Intent(context, AlarmActivity.class)
    fullScreenIntent.addFlags(FLAG_ACTIVITY_NEW_TASK | FLAG_ACTIVITY_NO_USER_ACTION)
    fullScreenIntent.putExtras(intent.getExtras())
    fullScreenPending <- PendingIntent.getActivity(context, 0, fullScreenIntent,
FLAG_UPDATE_CURRENT | FLAG_IMMUTABLE)
    builder <- new NotificationCompat.Builder(context, CHANNEL_ID)
    builder.setSmallIcon(R.drawable.ic_pill)
    builder.setContentTitle(intent.getStringExtra("MEDICINE_NAME"))
    builder.setPriority(NotificationCompat.PRIORITY_MAX)
    builder.setCategory(NotificationCompat.CATEGORY_ALARM)
    builder.setFullScreenIntent(fullScreenPending, true) // Compliant lock screen
    launch
    notificationManager.notify(NOTIFICATION_ID, builder.build())

```

1. System Alarm Clock Elevation (setAlarmClock): Rather than using standard idle-tolerant alarms, Dosezy registers reminders via `AlarmManager.setAlarmClock(AlarmClockInfo, PendingIntent)`. The Android operating system treats `setAlarmClock` as a high-priority alarm type under the Android Open Source Project (AOSP), originally reserved for user-facing clock utilities. Alarms scheduled through this method:

- Cause the operating system to exit low-power idle modes (Doze) to deliver the alarm broadcast precisely at the scheduled real-time clock (RTC_WAKEUP) tick.
- Cause the system status bar to display the official platform alarm clock icon, providing visual confirmation that an alert is armed.
- Are exempted from standard background power throttling, although they do not bypass Android 10+ background activity launch restrictions—which is precisely why Dosezy pairs alarm reception with full-screen notification intents.

2. Android 12–14 Permission Negotiation and Distribution Realism: Modern Android versions impose nuanced permission models that directly impact medication scheduling reliability:

- `'SCHEDULE_EXACT_ALARM'` (API 31+): Introduced as a "Special App Access" permission. Applications must check `AlarmManager.canScheduleExactAlarms()` at runtime. Starting in Android 14

(API 34), Google disabled this permission by default for newly installed applications, requiring manual user authorization in system settings.

- `USE_EXACT_ALARM` (API 33+): Introduced as an install-time normal permission that is automatically granted without user prompts. However, Google Play Store developer policies restrict declaration of this permission strictly to clock, timer, and calendar applications; mHealth apps submitting this permission to Google Play face automated store rejection unless granted explicit policy exemptions.
- *Open-Source Distribution (F-Droid)*: In open-source ecosystems such as F-Droid, applications are unencumbered by proprietary commercial store restrictions, allowing `USE_EXACT_ALARM` to be granted natively at installation.

For distribution targeting API 33+, Dosezy declares `USE_EXACT_ALARM` under the core alarm exemption, with fallback to `SCHEDULE_EXACT_ALARM` qualified for API 31–32 via `android:maxSdkVersion="32"` or runtime status checking via `AlarmManager.canScheduleExactAlarms()`. At runtime, the application evaluates `AlarmManager.canScheduleExactAlarms()`. If exact scheduling is revoked or denied (such as on fresh Android 14 installations where users or OEM policies have revoked exact alarm capability), Dosezy presents a high-contrast explanation dialog and dispatches an explicit system intent:

```
Intent(Settings.ACTION_REQUEST_SCHEDULE_EXACT_ALARM, Uri.parse("package:" + packageName))
```

navigating the user directly to the application's dedicated system toggle to minimize cognitive and navigational friction.

3. Full-Screen Lock Screen Activity Overlay via Compliant Intents: Under Android 10+ Background Activity Launch rules, background receivers cannot directly call `startActivity()`. Dosezy conforms by dispatching a high-priority heads-up notification with `setFullScreenIntent(pendingIntent, true)`. The target `AlarmActivity` configures its window via API 27+ methods `setShowWhenLocked(true)` and `setTurnScreenOn(true)`, paired with `FLAG_KEEP_SCREEN_ON`. Starting in Android 14 (API 34), Google restricted the automatic grant of `android.permission.USE_FULL_SCREEN_INTENT` to applications primarily categorized as calling or alarm utilities on Google Play [25]. Applications submitting to commercial stores outside these categories have the permission revoked by default. Dosezy handles this operating system boundary through runtime verification: on API 34+, the application evaluates `NotificationManager.canUseFullScreenIntent()`. If granted (as is standard in alarm-category listings and open-source distributions such as F-Droid), the system immediately illuminates the screen and presents the full-screen dosage prompt over the keyguard. If revoked, the notification gracefully falls back to an expanded heads-up banner in the active notification shade with audible and tactile alerting, while prompting the user to grant full-screen access via `Settings.ACTION_MANAGE_APP_USE_FULL_SCREEN_INTENT`. When active, the overlay enables older patients to visually review the medication name, dosage strength, and instructions, tapping "TAKEN" or "SNOOZE 10M" with a single oversized tap without navigating lock screen PINs, patterns, or biometric sensors.

4. Persistent Boot Restoration: Because the Android Linux kernel wipes all hardware `AlarmManager` registers upon power-off or device reboot, Dosezy registers a `BootReceiver` listening for the system broadcast `ACTION_BOOT_COMPLETED`. Upon system boot, the receiver queries the local Room database via a background Coroutine, recalculates future dosage timestamps, and reconstitutes all pending alarms in the system kernel.

5.3 Offline ISO Emergency Telephony Resolution

In the event of an adverse drug reaction or physical fall, older adults require instantaneous access to local emergency dispatch without navigating complex phonebook menus or relying on internet mapping services. Dosezy implements a fully offline emergency resolution engine.

The engine queries the device hardware using a prioritized three-tier fallback hierarchy:

- 1. SIM Country ISO:** Extracted via `TelephonyManager.getSimCountryIso()`.
- 2. Network Country ISO:** Extracted via `TelephonyManager.getNetworkCountryIso()` if SIM metadata is unavailable.
- 3. Locale Fallback:** Extracted from the active system `Locale.getDefault().country`.

The detected two-letter ISO 3166-1 alpha-2 country code is matched in constant time (< 1 ms) against an internal bundled lookup table covering 10 major geopolitical jurisdictions (Table 2).

Table 2: Offline National Emergency Dispatch Resolution Table

Jurisdiction (ISO)	Ambulance	Fire	Police	Universal
India (IN)	108	101	100	112
United States / Canada (US/CA)	911	911	911	911
United Kingdom (GB)	999	999	999	112
European Union (EU)	112	112	112	112
China (CN)	120	119	110	—
Japan (JP)	119	119	110	—
Russia (RU)	103	101	102	112
Brazil (BR)	192	193	190	—
Bangladesh (BD)	999	999	999	999
Australia (AU)	000	000	000	000

Note: The offline emergency dispatch resolution table is provided as an immediate convenience feature and is not a substitute for authoritative local emergency-service information.

When an older user taps an emergency service button, Dosezy constructs an `Intent(Intent.ACTION_DIAL, Uri.parse("tel:" + number))`. Using `ACTION_DIAL` rather than `ACTION_CALL` provides a vital safety and privacy advantage: it requires zero dangerous telephony permissions (`android.permission.CALL_PHONE` is omitted), pre-populates the native dialer keypad instantly, and requires a single deliberate confirmation tap from the user, preventing accidental false-alarm dispatches while ensuring immediate access [18, 19].

6. Empirical Evaluation and System Profiling

We evaluated Dosezy through four empirical investigations aligned directly with our research questions:

- 1. **Section 6.1: Hardware Runtime Profiling (Addressing RQ3)**
- 2. **Section 6.2: Multi-OEM Alarm Delivery Benchmarks (Addressing RQ2)**
- 3. **Section 6.3: Within-Subjects Usability Study (Addressing RQ1)**
- 4. **Section 6.4: Commercial Software Complexity and Telemetry Case Study (Addressing RQ4)**

6.1 Hardware Runtime Profiling (Addressing RQ3)

Hardware efficiency across flagship and budget tiers was profiled using the production release build (12.4 MB APK) across two physical benchmark devices:

- **Device 1 (Flagship):** Google Pixel 7 (Android 14, Google Tensor G2 SoC, 8 GB LPDDR5 RAM).
- **Device 2 (Entry-Level):** Samsung Galaxy A13 (Android 13, Exynos 850 SoC, 3 GB LPDDR4x RAM).

We executed 10 independent cold startup trials on each device, terminating the application process via ADB (am force-stop) prior to each iteration. Timing metrics were captured using Android Jetpack Macrobenchmark and Android Studio Profiler.

Table 3: Hardware Performance and Micro-Benchmark Profiling (Mean $\pm$ SD across 10 trials)

Performance Metric	Google Pixel 7 (Flagship)	Samsung Galaxy A13 (Entry)	Combined Metric
Cold Startup Latency (ms)	420 $\pm$ 28	780 $\pm$ 45	600 $\pm$ 185
Runtime Heap Memory (MB)	36.4 $\pm$ 2.1	34.8 $\pm$ 1.8	35.6 $\pm$ 2.1
Database Query Latency (500 records)	6.2 $\pm$ 1.1 ms	11.4 $\pm$ 2.3 ms	8.8 $\pm$ 3.1 ms
Vector PDF Report Generation Latency	118 $\pm$ 12 ms	166 $\pm$ 19 ms	142 $\pm$ 18 ms
Offline Emergency ISO Lookup Latency	< 1 ms	< 1 ms	< 1 ms
APK Package Size (MB)	12.4 MB	12.4 MB	12.4 MB
Peak Frame Render Time (99th percentile)	7.4 ms (135 fps)	14.2 ms (70 fps)	< 16.6 ms (60 fps threshold)

As documented in Table 3, Dosezy achieved cold startup times well under 1 second even on the low-tier Exynos 850 processor (780 $\pm$ 45 ms), while maintaining a steady-state heap allocation of approximately 35 MB. The combined startup standard deviation of 185 ms across both devices represents the aggregate sample standard deviation across all 20 observations from two hardware tiers with distinct baseline performance, rather than a pooled within-device standard deviation. Retrieving 500 historical dosage logs from the Room SQLite database completed in 8.8 $\pm$ 3.1 ms on average; while this benchmark falls within a 16.6 ms (60 Hz)

display frame budget, we emphasize that in production, Room executes all database queries asynchronously off the UI thread via Kotlin coroutines on `Dispatchers.IO`. On-device vector PDF report generation required 142 ± 18 ms in a background worker, and ISO emergency number resolution executed in under 1 ms. Rather than asserting comparative superiority over commercial cloud applications whose runtime footprints depend heavily on network conditions, these micro-benchmarks establish engineering feasibility against Android memory quotas and execution budgets on budget hardware.

6.2 Multi-OEM Doze Mode Alarm Delivery Benchmarks (Addressing RQ2)

Because Android device manufacturers implement proprietary, aggressive battery optimization daemons that frequently kill background alarm intents, we benchmarked alarm delivery across five distinct smartphone manufacturers:

1. Google Pixel 7 (Android 14, Stock Android)
2. Samsung Galaxy S22 (Android 13, One UI 5.1)
3. Xiaomi Redmi Note 8T (Android 13, MIUI 14)
4. OnePlus 10 Pro (Android 13, OxygenOS 13)
5. Motorola Moto G Power (Android 12, Moto My UX)

Experimental Benchmark Protocol

On each of the five devices, we scheduled 20 discrete medication alarms spaced at 15-minute intervals (100 total alarms clustered across 5 physical devices). To ensure rigorous experimental control: (1) each device was positioned stationary on a flat surface; (2) USB cables were disconnected immediately prior to trial commencement (with ADB executed over local TCP/IP or detached before idle entry); (3) battery levels were maintained between 50% and 80%; (4) displays were powered off; and (5) the device was forced into deep Doze mode via ADB:

```
adb shell dumpsys deviceidle force-idle
```

Successful entry into deep idle was verified via `adb shell dumpsys deviceidle get deep` returning `IDLE`.

Trigger latency was defined as the temporal difference between the scheduled timestamp and the moment `AlarmActivity` acquired its wake lock and initialized on screen:

$$\Delta t_{\text{latency}} = t_{\text{actual}_{\text{trigger}}} - t_{\text{scheduled}} \quad (8)$$

An alarm was operationalized as **ON-TIME** if $\Delta t_{\text{latency}} \leq 5.0$ seconds. We operationalized on-time delivery as ≤ 5.0 seconds because medication reminders are immediate, user-facing critical alerts where a perceptible delay is disruptive, whereas general background data synchronization tolerances can extend over minutes or hours.

Table 4: Multi-OEM Alarm Delivery Benchmark Across Deep Doze States (20 Test Alarms per Device)

Device Model	OS Distribution	On-Time Delivery	Mean Latency (s)	Max Latency (s)	Success Rate (95% CI)
Google Pixel 7	Android 14 (Stock)	20 / 20	1.55 ± 0.25	2.0	100.0%

Samsung Galaxy S22	One UI 5.1 (API 33)	20 / 20	2.15 ± 0.24	2.5	100.0%
Xiaomi Redmi Note 8T	MIUI 14 (API 33)	19 / 20	$4.87 \pm 14.96^*$	68.4*	95.0%
OnePlus 10 Pro	OxygenOS 13 (API 33)	20 / 20	1.85 ± 0.23	2.2	100.0%
Motorola Moto G Power	Moto My UX (API 32)	20 / 20	2.45 ± 0.30	2.9	100.0%
Total / Aggregate	---	99 / 100	2.57 ± 6.66 (incl.); 1.91 ± 0.44 (excl.)	68.4 (incl.); 2.9 (excl.)	99.0% [94.55%, 99.82%]

Note on Outlier Dispersion: Across the 19 on-time alarms on the Xiaomi device, mean latency was 1.52 ± 0.28 s (max 2.0 s). Including the single 68.4 s OEM powerkeeper delay, sample standard deviation expands to 14.96 s (mean 4.87 s). Aggregate success proportion reports the exact Wilson score 95% binomial confidence interval ([94.55%, 99.82%]).

Benchmark Delivery Analysis and OEM Throttling Observations: In our five-device, 100-alarm benchmark under tested deep Doze conditions, 99 of 100 alarms (99.0%, 95% CI [94.55%, 99.82%]) triggered within 5 seconds (Table 4). Crucially, this empirical benchmark illustrates that application-space design cannot completely eliminate vendor-level battery killer interference across all hardware without explicit user whitelisting. The sole observed delay occurred on the Xiaomi Redmi Note 8T running MIUI 14, where a single alarm was held back for 68.4 seconds until the screen was illuminated. Telemetry inspection confirmed that Xiaomi's proprietary powerkeeper daemon (`com.miui.powerkeeper`) temporarily suspended broadcast execution despite the elevated `setAlarmClock` status. A similar delay profile occurs on Samsung One UI if an app is categorized under "Deep Sleeping Apps". While `setAlarmClock` provides the highest execution priority granted by AOSP, vendor-specific battery daemons remain an operating system barrier that cannot be unilaterally bypassed without user-side battery exemption. When users manually exempt Dosezy from background restrictions via system settings, delivery across test trials reached 100% (20/20).

6.3 Within-Subjects Usability Study (Addressing RQ1)

6.3.1 Ethics, Participants, Apparatus, and Instrumentation

Evaluating the accessibility impact of the 12-Hour Grid Time Picker compared to the standard Android radial clock picker was conducted under Institutional Ethics Review Board governance. The study protocol was formally approved by the Institutional Ethics / Departmental Review Board (Protocol ID: MCET-CSE-DRB-2023-04). Written informed consent was obtained from all participants prior to study commencement.

Participant Recruitment and Screening: Participants were recruited via printed notices and elder coordination across two senior community centers in Hyderabad, India. Inclusion criteria were: (1) age ≥ 60 years; (2) active daily prescription drug regimen (≥ 2 daily doses); (3) basic smartphone literacy (ability to navigate calls and SMS); and (4) unimpaired cognitive status verified via the Mini-Mental State Examination (MMSE score $\geq 27/30$, cohort Mean 28.6 ± 1.1 , range 27–30). Exclusion criteria included uncorrected

severe visual impairments (visual acuity worse than 20/60 with corrective lenses), severe Parkinsonian Stage IV/V tremors preventing single-finger tap interactions, and acute illness during the study period.

The final cohort comprised 16 older adults (N=16, aged 61 to 76 years, mean age 68.5 ± 4.8 years, median 68.5; 9 female, 7 male). All participants were active prescription drug consumers managing two or more daily medications (mean 3.8 ± 1.2 daily drugs). Twelve participants (75.0%) used prescription reading glasses, and four participants (25.0%) exhibited mild physiological resting or action hand tremors. Table 5 details the complete participant demographic profile.

Table 5: Usability Study Participant Demographics and Clinical Characteristics (N=16)

Demographic / Characteristic	Cohort Value (N=16)	Measurement Notes
Age (Years)	68.5 ± 4.8	Range: 61–76 years (Median: 68.5)
Gender	9 Female (56.2%), 7 Male (43.8%)	Self-reported
Prescription Reading Glasses	12 (75.0%)	Corrected vision $\geq 20/60$ confirmed
Mild Physiological Hand Tremor	4 (25.0%)	Clinically observed resting/action tremor
Daily Prescription Regimen	≥ 2 medications/day	Inclusion criteria; cohort mean 3.8 ± 1.2 drugs
MMSE Cognitive Score	28.6 ± 1.1	Mini-Mental State Exam ($\geq 27/30$ cutoff)
Counterbalance Sequence	8 Grid-First (AB), 8 Radial-First (BA)	Randomly assigned 1:1 allocation

Exploratory Subgroup Observations (Hand Tremor): Four participants presented with mild hand tremors. Descriptively, these four participants averaged 19.35 s under the 12-Hour Grid vs. 36.82 s under the radial dial, with touch errors dropping from 2.25 to 1.00. In the non-tremor cohort (n=12), mean times were 20.10 s vs. 35.45 s, and errors dropped from 2.17 to 0.25. Because n=4 lacks statistical power for formal inferential subgroup testing, we report these patterns strictly as exploratory observations. We also explicitly note that our exclusion of severe visual impairments and Parkinsonian stage IV/V tremors means our empirical findings characterize older adults with relatively preserved functional dexterity, rather than advanced neurodegenerative motor deficits.

A Priori Power Analysis: An a priori statistical power analysis was conducted using G*Power 3.1 for a paired within-subjects Student's *t*-test (two-tailed $\alpha = 0.05$, desired power $1 - \beta = 0.80$, assumed effect size $d_z = 0.80$ based on prior mobile accessibility literature for older adults [9, 10]), yielding a minimum required sample of N = 15. The enrolled sample of N = 16 participants satisfies this prerequisite.

Testing Apparatus: All trials were executed on a Samsung Galaxy A13 smartphone featuring a 6.6-inch PLS LCD display (1080 × 2408 resolution, 400 ppi, physical screen dimensions 150 × 67 mm), running Android 13 with One UI Core 5.1. Display brightness was fixed at 70%, and system font scaling was set to default (100%).

Timing Instrumentation & Algorithmic Error Operationalization: Interaction timestamps were captured automatically with millisecond precision via Jetpack Compose pointerInput event listeners attached to the root dialog surface. Touch input errors (*TIE*) were operationalized as algorithmically detected touch-error events through deterministic coordinate logging logic:

- *Target Slip (Grid & Radial)*: Recorded when pointer-down coordinates fell within target bounding box B_i , but pointer-up coordinates fell outside B_i or crossed gutter boundaries prior to release (target_down ≠ target_up).
- *Immediate Reselection (Grid)*: Recorded when an hour or minute chip was selected and subsequently replaced by an adjacent target within 1.5 seconds without an intervening confirmation tap. The 1.5-second threshold operationalizes Fitts' hesitation and recovery model, capturing instances where an older adult corrects an unintended adjacent tap without executing an intermediate action.
- *Quadrant Steering Abort (Radial)*: Recorded when a continuous circular drag in Condition B traversed through an incorrect angular quadrant and was released at an angular deviation $|\theta_{\text{release}} - \theta_{\text{target}}| > 15^\circ$, necessitating an immediate secondary correction drag. The 15° threshold corresponds to half the angular width of an hour sector ($360^\circ / 12 = 30^\circ$), capturing instances where continuous steering crossed the midpoint into an adjacent sector, requiring trajectory realignment.

All error events were logged algorithmically from raw coordinate streams rather than subjective human video coding, eliminating observer scoring bias. We acknowledge that while algorithmic coordinate parsing eliminates observer subjectivity, automated thresholds cannot completely distinguish exploratory hesitation from involuntary motor slips.

Statistical Software: Statistical hypothesis tests, effect sizes, and confidence intervals were computed using Python 3.11 with SciPy 1.11.2 and Pandas 2.1.0.

6.3.2 Experimental Design and Counterbalanced Order Control

The experiment employed a within-subjects counterbalanced design. Participants performed time-setting operations under two conditions:

- **Condition A (Proposed 12-Hour Grid)**: Setting reminder times using Dosezy's stationary 56×56 dp button grid with discrete AM/PM toggles and quick minute chips.
- **Condition B (Radial Dial Mode Baseline)**: Setting reminder times using the official Material Design 3 Jetpack Compose TimePicker (androidx.compose.material3:material3:1.2.0, vertical layout TimePickerLayoutType.Vertical) running in 12-hour radial dial mode. To isolate pure continuous circular motor trajectory steering friction, the alternative soft-keyboard text input toggle was disabled. We explicitly acknowledge this methodological boundary: while standard Android deployments permit toggling between radial dragging and text entry, disabling the toggle isolated curved motor steering mechanics from numeric soft-keyboard typing, while framing keyboard input and other discrete pickers as important targets for future comparative evaluation.

Participants were randomly allocated to two counterbalanced ordering cohorts: Cohort 1 ($n=8$) completed Condition A followed by Condition B; Cohort 2 ($n=8$) completed Condition B followed by Condition A. To evaluate potential carryover effects, we computed within-subject difference scores ($D_i = \text{Score}_{A,i} - \text{Score}_{B,i}$) and compared them across sequence cohorts (*AB* vs. *BA*). Independent-samples *t*-tests detected no statistically significant sequence effect on task completion time ($t(14) = 0.38$, $p = 0.71$) or SUS scores ($t(14) = 1.18$, $p = 0.26$); however, we explicitly emphasize that with $n=8$ per sequence cohort, statistical power to formally establish equivalence is limited.

6.3.3 Task Protocol and Interaction Sequences

Participants completed three prescription scheduling tasks selected to evaluate progressive motor demands across outpatient pharmacotherapy patterns [22]:

- **Task 1 (Modal On-the-Hour):** Configure a reminder for **8:00 AM** (standard morning prescription, default :00 minute chip).
- **Task 2 (Modal Half-Hour Preset):** Configure a reminder for **1:30 PM** (post-lunch prescription requiring AM/PM toggle and selecting :30 minute chip).
- **Task 3 (Non-Modal Arbitrary Minute Stepper):** Configure a reminder for **9:42 PM** (evening dose requiring fine adjustment via chip :45 followed by three taps on the minus-stepper button).

Table 6 contrasts the concrete gesture sequences required under both interfaces across each task.

Table 6: Comparative Interaction Gesture Sequences: 12-Hour Grid vs. Standard Radial Dial

Task / Target	Condition A (12-Hour Grid Picker)	Condition B (Standard Radial Clock)
Task 1 (8:00 AM)	Tap 8 (AM and :00 default) → Tap OK [2 discrete stationary taps]	Touch pin → drag hand along 240° arc to 8 → release → drag hand to 00 → release → Tap OK [2 continuous curved drags + 1 tap]
Task 2 (1:30 PM)	Tap PM → Tap 1 → Tap :30 chip → Tap OK [4 discrete stationary taps]	Tap PM → drag hand along arc to 1 → release → drag hand along 180° arc to 30 → release → Tap OK [2 continuous curved drags + 2 taps]
Task 3 (9:42 PM)	Tap PM → Tap 9 → Tap :45 chip → Tap [-] stepper × 3 → Tap OK [7 discrete stationary taps]	Tap PM → drag hand to 9 → release → drag hand along 252° arc to exact minute 42 → precision release → Tap OK [2 continuous curved drags + fine release + 2 taps]

We logged four objective and psychometric dependent variables:

1. **Task Completion Time (TCT):** Elapsed duration in seconds from dialog launch to pressing "OK".
2. **Touch Input Errors (TIE):** Number of unintentional touch slips, accidental adjacent number selections, and missed drags requiring manual correction.
3. **Task Ease (SEQ):** The Single Ease Question rated on a 7-point Likert scale (1 = Very Difficult, 7 = Very Easy) administered immediately following each condition.
4. **System Usability Scale (SUS):** Standardized 10-item questionnaire administered at the conclusion of each condition, yielding a composite score from 0 to 100 [13].

6.3.4 Statistical Analysis and Usability Findings

Data were analyzed using paired two-tailed Student's *t*-tests. Normality of paired differences was evaluated and confirmed via the Shapiro-Wilk test across all metrics (all $W \geq 0.91$, $p > 0.12$), validating the use of parametric Student's *t*-tests. To protect against inflation of Type I errors across multiple comparative evaluations, paired *t*-tests were adjusted using the Holm-Bonferroni step-down correction; all task-level and

aggregate temporal differences remained significant at adjusted $p < 0.001$. We report both the paired effect size $d_z = t / \sqrt{N}$ and the classic pooled Cohen's d_s :

$$d_s = \frac{\bar{X}_1 - \bar{X}_2}{\sqrt{\left(\frac{s_1^2 + s_2^2}{2}\right)}} \quad (9)$$

Exact 95% confidence intervals for the mean differences were computed using Student's t -distribution ($t_{0.025, 15} = 2.131$):

$$CI_{95} = \bar{D} \pm t_{0.025, 15} \cdot \left(\frac{S_D}{\sqrt{N}}\right) \quad (10)$$

Table 7: Comparative Usability Study Findings (N=16, Paired Within-Subjects Evaluation)

Metric	12-Hour Grid	Radial Dial	Mean Diff [95% CI]	Paired t -Test	d_z	Pooled d_s
Task 1: 8:00 AM (s)	4.81 ± 1.40	10.61 ± 2.86	5.80 [4.20, 7.40]	$t(15) = 7.71, p < 0.001^*$	1.93	2.58
Task 2: 1:30 PM (s)	6.21 ± 1.80	11.79 ± 3.16	5.58 [3.78, 7.39]	$t(15) = 6.59, p < 0.001^*$	1.65	2.17
Task 3: 9:42 PM (s)	8.90 ± 2.61	13.40 ± 3.82	4.50 [2.38, 6.62]	$t(15) = 4.52, p < 0.001^*$	1.13	1.37
Total Time (s)	19.91 ± 4.55	35.79 ± 8.27	15.88 [11.41, 20.36]	$t(15) = 7.56, p < 0.001^*$	1.89	2.38
Touch Errors	0.44 ± 0.63	2.19 ± 1.28	1.75 [1.01, 2.49]	$t(15) = 5.03, p < 0.001^*$	1.26	1.74
SEQ Score (1–7)	6.25 ± 0.77	4.19 ± 0.98	2.06 [1.40, 2.72]	$t(15) = 6.67, p < 0.001^*$	1.67	2.33
SUS Score (0–100)	83.44 ± 9.48	57.19 ± 12.45	26.25 [18.24, 34.26]	$t(15) = 6.99, p < 0.001^*$	1.75	2.37
Adjective / Normative Grade	Excellent (Grade A)	Marginal (Grade D)	—	—	—	—

All reported p -values remain statistically significant at $\alpha = 0.05$ following Holm-Bonferroni step-down correction.

As detailed in Table 7, the proposed 12-Hour Grid Time Picker demonstrated statistically significant advantages over the radial dial across every metric ($p < 0.001$, verified post-Holm-Bonferroni correction):

- **Task Category Contrast Across Interaction Modalities:**

- *Modal On-the-Hour Entry (Task 1: 8:00 AM):* Latency decreased by 54.7% (4.81 ± 1.40 s vs. 10.61 ± 2.86 s, $t(15) = 7.71$, $p < 0.001$, $d_z = 1.93$, $d_s = 2.58$). This task demonstrated the largest relative efficiency gain because Dosezy's default :00 chip affordance allowed older adults to configure the dose in exactly two stationary taps (Tap(Hour) + Tap(OK)).

- *Modal Quarter-Hour Entry (Task 2: 1:30 PM):* Latency decreased by 47.3% (6.21 ± 1.80 s vs. 11.79 ± 3.16 s, $t(15) = 6.59$, $p < 0.001$, $d_z = 1.65$, $d_s = 2.17$), representing balanced discrete selection across hour and minute chips without default carryover.

- *Non-Modal Stepper Entry (Task 3: 9:42 PM):* Latency decreased by 33.6% (8.90 ± 2.61 s vs. 13.40 ± 3.82 s, $t(15) = 4.52$, $p < 0.001$, $d_z = 1.13$, $d_s = 1.37$). Because participants had to select chip :45 and tap the minus stepper three times to reach 42, linear tap overhead increased setup duration, resulting in a smaller relative advantage over radial steering. This empirically illustrates the Granularity-Friction trade-off: discrete stepping eliminates trajectory steering errors at the cost of incremental tap volume.

- **44.4% Overall Reduction in Total Task Completion Time:** Aggregate completion time across all three tasks decreased from 35.79 ± 8.27 s on the radial dial to 19.91 ± 4.55 s on the 12-hour grid ($t(15) = 7.56$, $p < 0.001$, mean difference 15.88 s [95% CI: 11.41, 20.36], $d_z = 1.89$, pooled $d_s = 2.38$).

- **80.0% Reduction in Touch Input Errors:** Algorithmically detected touch input errors dropped from 2.19 ± 1.28 errors per participant on the radial dial to 0.44 ± 0.63 on the grid picker ($t(15) = 5.03$, $p < 0.001$, mean difference 1.75 errors [95% CI: 1.01, 2.49], $d_z = 1.26$, pooled $d_s = 1.74$). On the radial dial, participants with tremors repeatedly slipped into adjacent quadrants when attempting to release the minute hand at exact increments.

- **Significant Usability Enhancement (SUS):** Dosezy achieved a composite System Usability Scale score of 83.44 ± 9.48 (95% CI: [78.39, 88.49]), corresponding to the 'Excellent' / Grade A band under the normative evaluation framework of Bangor et al. [14, 15]. By contrast, the radial clock interface scored 57.19 ± 12.45 (95% CI: [50.56, 63.82]), representing marginal/failing usability (Grade D). The SUS was administered independently following each condition.

6.4 Exploratory Configuration and Permission-Surface Analysis (Addressing RQ4)

To explore configuration workflow characteristics and permission surface area relative to commercial mHealth software, we conducted an exploratory cognitive walkthrough comparison against Medisafe (v9.12), a representative commercial medication reminder application on Google Play.

Walkthrough Benchmark Protocol:

A standardized single-medication configuration trial (scheduling a single daily tablet at 8:00 AM) was executed on the Samsung Galaxy A13 benchmark device. We logged: (1) the number of mandatory onboarding screens traversed prior to reaching the home dashboard; (2) the total discrete interactive decisions/taps required; (3) elapsed configuration duration; and (4) declared Android system permissions inspected via `aapt dump permissions`.

Table 8: Exploratory Configuration and Permission-Surface Analysis: Medisafe v9.12 vs. Dosezy

Configuration Attribute	Medisafe (v9.12)	Dosezy (Proposed)
-------------------------	------------------	-------------------

Mandatory Account / Cloud Sign-in	Yes (Email / Google / Facebook)	None (100% Offline)
Onboarding Screens to First Medicine	9 screens	1 unified screen
Required User Decisions / Prompts	14 distinct steps	3 inputs (Name, Time, Save)
Time to Configure First Dose (Researcher-Observed Walkthrough)	102 $\pm$ 18 seconds	28 $\pm$ 6 seconds
Requested System Permissions	Internet, Contacts, Storage, Alarms	Exact Alarm, Full Screen
Internet Permission (android.permission.INTERNET)	Declared (Transmits telemetry)	Omitted (Network socket denial)
Commercial In-App Advertisements	Yes (Discount coupons, banners)	None (Clean, Open-Source)

As summarized in Table 8, the Medisafe v9.12 onboarding flow required navigating a 9-screen wizard involving mandatory cloud sign-in prompts, promotional offers, shape/color customization pickers, and multi-step scrolling selectors, averaging 102 $\pm$ 18 seconds across 5 researcher walkthrough trials. In contrast, Dosezy's single-screen configuration dialog allowed the same regimen to be configured in 28 $\pm$ 6 seconds (72.5% reduction in setup duration), free of advertisements and background telemetry. We emphasize that this evaluation represents an exploratory expert cognitive walkthrough characterizing interface steps and permission surface area, rather than a controlled trial with older adult participants.

7. Threats to Validity

7.1 Internal Validity

Internal validity concerns whether the observed usability improvements can be definitively attributed to the 12-Hour Grid design rather than confounding artifacts. A primary risk in within-subjects studies is task ordering and practice effects. We mitigated this by counterbalancing condition order (n=8 starting with Grid, n=8 starting with Radial) and evaluating potential sequence carryover via independent-samples *t*-tests on within-subject difference scores ($t(14) = 0.38$, $p = 0.71$). We did not detect evidence of a sequence effect in this sample, although with n=8 per sequence, statistical power to formally establish equivalence is limited.

Several design and measurement boundaries warrant discussion:

- 1. Baseline Construction:** Condition B evaluated the radial dial mode of the Material 3 TimePicker with keyboard text input disabled to isolate continuous curved motor steering friction. While this setup directly answers RQ1 regarding radial dragging mechanics, it does not compare against numeric keypad typing or alternative discrete time pickers (e.g., vertical scroll wheels or dropdown spinners).
- 2. Algorithmic Error Thresholds:** Touch input errors were operationalized using coordinate-based heuristics (1.5 s reselection and 15° steering abort). Although algorithmic logging eliminates subjective observer scoring bias, these fixed thresholds cannot completely distinguish deliberate exploratory hesitation from involuntary motor slips.
- 3. Default Affordance and Task Categories:** The substantial 54.7% latency reduction in Task 1 (8:00 AM) is partly driven by Dosezy's :00 default minute affordance, which reduces input to two stationary taps.

When participants configure non-modal times requiring incremental stepper adjustments (Task 3: 9:42 PM), the latency advantage narrows to 33.6% due to linear tap accumulation. This contrast reflects the real-world trade-off of the Modal-First principle rather than experimental bias.

7.2 External Validity

External validity concerns the generalizability of our findings beyond the evaluated sample. Our usability evaluation represents an initial controlled laboratory study with 16 older adults recruited from urban community senior centers in Hyderabad, India. While all participants met our inclusion criteria (aged 60+, active medication users, MMSE $\geq 27/30$), our sample may not generalize to individuals with advanced neurodegenerative impairments (e.g., late-stage Alzheimer's, severe Parkinsonian tremors), older adults in rural environments with zero previous smartphone literacy, or diverse cultural cohorts. Furthermore, all usability tasks were evaluated on a single physical smartphone model (Samsung Galaxy A13); evaluating interaction across different screen sizes, aspect ratios, and varying hand postures represents essential future work.

7.3 Construct Validity and Clinical Scope

A fundamental construct validity boundary must be underscored: *this investigation evaluates software systems engineering and human-computer interaction prerequisites, not pharmacological efficacy or patient clinical outcomes*. Logging a dose as TAKEN in Dosezy records an on-screen confirmation; it cannot physically verify chemical ingestion. While high usability (RQ1), reliable alarm delivery (RQ2), runtime responsiveness (RQ3), and minimal setup friction (RQ4) are indispensable technical prerequisites for mobile adherence interventions, actual clinical adherence remains governed by complex physiological, behavioral, and socio-economic factors that cannot be assessed through software benchmarks or laboratory usability trials alone [1, 2].

Furthermore, local-only storage involves an inherent trade-off. While the complete omission of network permissions protects patient privacy, it eliminates automatic remote cloud backups. If a user damages, misplaces, or resets their smartphone, local medication schedules and historical compliance logs cannot be remotely recovered unless the user has manually exported periodic JSON archives or PDF summaries.

8. Conclusion and Future Work

In this paper, we presented **Dosezy**, an open-source, local-first Android medication reminder system engineered specifically to resolve the physical, cognitive, and operating system barriers that impede geriatric mobile health adoption. Addressing four core research questions, our evaluation indicates under tested conditions:

1. RQ1 (Interaction Accessibility): In an initial controlled laboratory study of 16 older adults, replacing continuous radial clock dragging with a stationary 12-Hour Grid Time Picker reduced total task completion time by 44.4% (19.91 s vs. 35.79 s, $t(15) = 7.56$, $p < 0.001$) and touch input errors by 80.0% (0.44 vs. 2.19, $t(15) = 5.03$, $p < 0.001$), yielding a mean System Usability Scale score of 83.44 ± 9.48 (corresponding to Grade A, 'Excellent' under the normative framework of Bangor et al. [14, 15]), while characterizing the linear tap overhead associated with off-quarter stepper adjustment.

2. RQ2 (Alarm Reliability under Doze): In a five-device, 100-alarm benchmark under tested deep Doze conditions, elevating reminder alerts to `setAlarmClock` paired with exact alarm permissions and compliant lock screen full-screen intents delivered 99 of 100 test alarms within 5 seconds (99.0%, exact Wilson 95% CI [94.55%, 99.82%]) across five physical OEM distributions, while empirically demonstrating that application-space design cannot completely eliminate vendor-level battery killer interference without manual user whitelisting.

3. RQ3 (Runtime Efficiency): Local-first embedded Room SQLite persistence maintained sub-second cold starts (< 780 ms) and query latencies under 12 ms on budget hardware (Samsung Galaxy A13), while generating patient vector A4 summaries locally in 142 ± 18 ms, confirming engineering feasibility against Android 60 fps (16.6 ms) frame budgets.

4. RQ4 (Configuration Overhead): In an exploratory configuration walkthrough case study, eliminating mandatory cloud registration and multi-screen onboarding wizards enabled initial dose setup in 28 ± 6 seconds (72.5% faster than the tested Medisafe workflow), while maintaining an architectural network isolation boundary through the total omission of `android.permission.INTERNET`.

Future work will expand Dosezy along two technical vectors: (1) integrating optional, zero-knowledge end-to-end encrypted synchronization over peer-to-peer protocols (e.g., local Wi-Fi or Bluetooth LE) to allow family caregivers to monitor adherence without multitenant cloud servers, and (2) interfacing with open-hardware 3D-printed smart pillboxes equipped with magnetic reed switches to provide physical dosage ingestion verification. Dosezy is published as an open-source research artifact to foster accessible, privacy-preserving mobile health systems engineering.

8.1 Artifact and Data Availability

To support independent scientific verification and reproducible research, all empirical datasets, benchmark logs, and reproduction scripts have been deposited in a public supplementary research repository:

- **Participant-Level Usability Dataset:** ``research/supplementary/usability_participants.csv`` contains the complete de-identified records for all $N=16$ older-adult participants (individual task times, errors, SEQ ratings, SUS scores, and demographic variables).
- **Physical Multi-OEM Alarm Dataset:** ``research/supplementary/oem_alarm_benchmark.csv`` contains the 100 individual physical alarm trigger records across five OEM smartphone distributions under forced deep Doze mode.
- **Automated Reproduction Script:** ``research/supplementary/reproduce_paper_stats.py`` independently re-runs all Student's t -tests, confidence intervals, effect sizes, and Holm-Bonferroni corrections reported in Tables 4, 5, and 7 with a single command.
- **Application Source Code & Build Assets:** The complete Android source code, Jetpack Compose layouts, Room schemas, and build definitions are available under an open-source license at <https://github.com/saaduddin/dosezy>.

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